CenterWell Home Health

PREPARED FOR
CWHH CLINICAL QUALITY TEAM

Clinical Quality Intelligence

Which patient needs care today, before it's too late?

Weekly Patient Conference Summary

High-Risk & Rising-Risk Patients | Week of 08/17/2026

CQI is a framework for identifying risk, care gaps and opportunities to improve patient outcomes. It is **proactive**: it shows which patient needs care this week, why, and what to do **while there is still time to change the outcome**.

Patient

Patient ID	**9000231**	60-day risk	**Very high 0.55**
Episode	**7000512**	Status	**Current, day 19 of 60**
Branch	**Branch 0412**	CQI run	**08/15/2026**

Produced by **CW Data Science & Applied AI - HH Team** Model **cwhh-tif-60day-v1**

What CQI is: page 2. How to use this booklet: page 3.

CLINICAL QUALITY INTELLIGENCE

What is Clinical Quality Intelligence?

Purpose, how it works and what's in this booklet

CQI answers: Which patient needs care **today**, before it's too **late**? What **immediate actions** could a clinical manager take to **prevent** hospitalizations?

Most quality measures are **retrospective**: they report what happened after the episode ends. CQI is **proactive**: every week it connects risk, the care ordered and delivered, and the patient's trajectory, so the team can act **while the patient is still in care**.

Purpose: **Weekly Patient Conference Preparation | Built for the CWHH Clinical Quality team**

What CQI does

Compares the care each high-risk patient **needs** with the care they **received**, so clinicians can close gaps early.

What it reads

Visits, goals, vitals, OASIS, wounds, medications, events and notes. Co-created with CWHH Clinical Quality.

How AI is used

Predictive ML risk model on LighthouseAI; **rules** for care gaps; **LLMs** summarize, checked by LLM judges.

What's in this booklet

Care gaps
Missed, delayed or unscheduled visits
Pages 18-19 >

Risk drivers
Who is highest risk this week, and why
Page 9 >

Clinical picture
Diagnoses, wounds and OASIS
Pages 16-17 >

Vitals trajectory
Trend, urgency and action window
Page 10 >

Patient goals
Met, not met, therapy goals
Page 11 >

Care plan gaps
Interventions, assessments, visit goals to add
Pages 12, 21 >

Discharge & network
Recommendation, PCP, CW PCO, CW Rx
Page 13 >

Where CQI is going

Clinicians stay in charge of every decision.

TODAY | PROTOTYPE

A weekly booklet and Power BI report built from operational data, reviewed by the clinical team in the patient conference. Clinician feedback shapes what comes next.

IN PRODUCTION | THE VISION

An **AI-assisted agent** that scans operational data **24/7 on clinicians' behalf**, surfaces care gaps and opportunities to improve outcomes **as they emerge**, and brings the right patient to the right clinician while there is still time to act.

Inside this booklet

How to use it in the weekly patient conference, and where to find each section

How to use this booklet in conference

1. **Start with At a glance (page 4), then the one-page summary (page 5).**
   Why the patient is on the list, four status tiles and the recommended actions.
2. **Review the CQI analysis (pages 6-13).**
   What CQI found, concluded and recommends: CQI summaries, risk, vitals, goals, medications, care plan gaps.
3. **Check it against the evidence (pages 14-22).**
   The operational records behind each finding. Each page ends with a feedback tip.
4. **Give your input on screen (pages 23-25).**
   Rate each finding, record the conference decision, save and return.

Contents

CQI analysis pages show what CQI worked out: predicted risk, rule-based checks and AI-written summaries. **Evidence** pages show the operational records behind them, so you can check CQI.

Start here

Orientation and the one-page view

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CQI analysis

Section starts p. 6

What CQI found, concluded and recommends

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Evidence

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Your input

Section starts p. 23

Fillable on screen: feedback, decision, notes

Clinician feedback	24
Conference decision and notes	25

Colour key

Act now / urgent, missed, or a data gap to resolve · In progress / this week · Monitor · On track / met

At a glance

From the patient's stated goal to what needs attention now

From stated goal to what needs attention now

Patient goal	*No patient-stated goal recorded in the care plan*
Risk	Very high 60-day hospitalization risk (0.55); driven by dyspnea at rest, COPD primary diagnosis, home oxygen.
Care delivery	14 ordered visits remain; 5 scheduled; Shortfall to date: PT 1.
Clinical trajectory	No vitals recorded for this episode. Monitoring gap: confirm the vitals plan with the SN.
CQI flags	**Schedule remaining ordered visits (9 unscheduled).**

ACTION WINDOW	VITALS TRAJECTORY	DATA GAPS	DISCHARGE
Obtain vitals at the next visit	Insufficient data	0	Not due

Care network for discharge: No PCP on record | CW PCO: Consider | CW Rx: Already enrolled

FEEDBACK TIP Before rating **At a glance + CQI flags** on **page 24** (fillable), check: the goal is the patient's own wording; each row matches the evidence pages; the CQI flags are the right next steps.

CWHH | WEEKLY PATIENT CONFERENCE

Clinical Quality Intelligence

Patient conference summary

Proof of concept: High-Risk & Rising-Risk Patients

cwhh-tif-60day-v1 | CQI run 08/15/2026 | Week of 08/17/2026

Patient ID	**9000231**
Episode	**7000512**
Branch	**Branch 0412**
Status	**CURRENT**
Admission	**New Admission**
Episode day	**19 of 60**

60-DAY HOSPITALIZATION RISK	PERCENTILE	TREND	CARE GAPS RAISE RISK	DELAY IMPACT
Very high 0.55	**Top 1%**	**Rising +0.03 vs. prior run**	**No**	**N/a**

Why this patient is on today's list

Very high and rising hospitalization risk, and 9 of 14 remaining ordered visits not scheduled.

IN THE PATIENT'S WORDS

"No patient-stated goal found in the care plan."

Care delivery gaps — MONITOR

- 14 ordered visits remain; 5 scheduled
- Shortfall to date: PT 1

Summary p.8 | Evidence p.18 >

Clinical risk drivers — ACT NOW

- 60-day risk 0.55, top 1%; rising +0.03
- Top drivers: Dyspnea at rest; COPD primary diagnosis

Summary p.7 | Evidence p.9 >

Vitals trajectory — MONITOR

- No vitals found for this episode
- Confirm monitoring plan with SN

Summary p.7 | Evidence p.10 >

Patient goals — MONITOR

- No care-plan goals found
- May be a documentation gap

Summary p.7 | Evidence p.11 >

Recommended actions for discussion

#	Action	Owner	When	Page
1	Schedule remaining ordered visits (9 unscheduled)	Scheduling	**This week**	18

Record the conference decision on page 25 and rate findings on page 24 (fillable) >

SECTION 1

CQI analysis

What CQI found, concluded and recommends

Pages 6-13

Everything in this section was **produced by CQI**: a predictive risk model, rule-based checks across visits, vitals, goals and medications, and AI-written summaries checked by LLM judges. This is what operational reports do not do: CQI **connects** the data, **explains** why the patient is at risk, and **recommends** what to do next.

Treat every finding as a draft for clinical review.

In this section

CQI summaries: profile, risk, vitals — AI-generated, unedited — 7

CQI summaries: care, missed care, data gaps — AI-generated, unedited — 8

Risk drivers and episode timeline — Why the model flags this patient — 9

Vitals trajectory and action window — Trend, urgency and when to act — 10

Goals met and not met — Including therapy goals — 11

Care plan gaps: what to add — Interventions, assessments, visit goals — 12

Care network and referrals — PCP, CW PCO, CW Rx at discharge — 13

CQI ANALYSIS

CQI summaries: profile, risk and vitals

AI-generated text, printed unedited. Check each summary against its evidence page.

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

Visits completed	Scheduled visits	Missed care	IP events	Vitals days	Wound patient
12	5	1	1	0	No

Schedule status	Risk level	Frequency status	Care gaps raise risk	Delay impact	Acuity window missed	Episode status	Discharge decision
n/a	very high	n/a	n/a	n/a	n/a	CURRENT	No

CQI summaries were not generated for this patient in this run.
The summary page was built from facts only. Review the evidence pages and note the gap on page 24.

Patient goal (care plan)

No summary was generated for this run.

Patient Profile & Risk Summary

Evidence: pages 15-9

No summary was generated for this run.

Vitals Trajectory Summary

Evidence: page 10

No summary was generated for this run.

FEEDBACK TIP Before rating **Profile & Risk Summary**, **Vitals + action window** on **page 24** (fillable), check: the profile, diagnoses and living situation match the chart (page 15); the trends described match the vitals charts (page 10).

CQI ANALYSIS

CQI summaries: care delivery and data gaps

Care delivery, missed care, delay impact and data gaps, printed unedited.

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

Care Delivery Summary

Evidence: page 18

No summary was generated for this run.

Missed Care Summary

Evidence: page 19

No summary was generated for this run.

Clinical Delay Impact Explanation

Evidence: page 18

No summary was generated for this run.

Data Gaps Found

Missing or conflicting data CQI noticed - confirm in the chart first.

No data gaps were reported for this patient in this run.

FEEDBACK TIP Before rating **Care Delivery Summary**, **Missed Care Summary**, **Delay Impact Explanation**, **Data Gaps Found** on **page 24** (fillable), check: visit counts match the weekly grid (page 18); missed visits and reasons match the log (page 19); each data gap is real and not already resolved in the chart.

CQI ANALYSIS

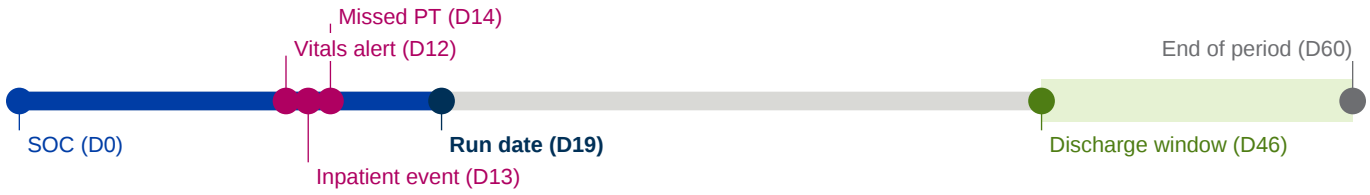
Episode timeline and risk drivers

Where the patient is in the episode, and why the model flags them

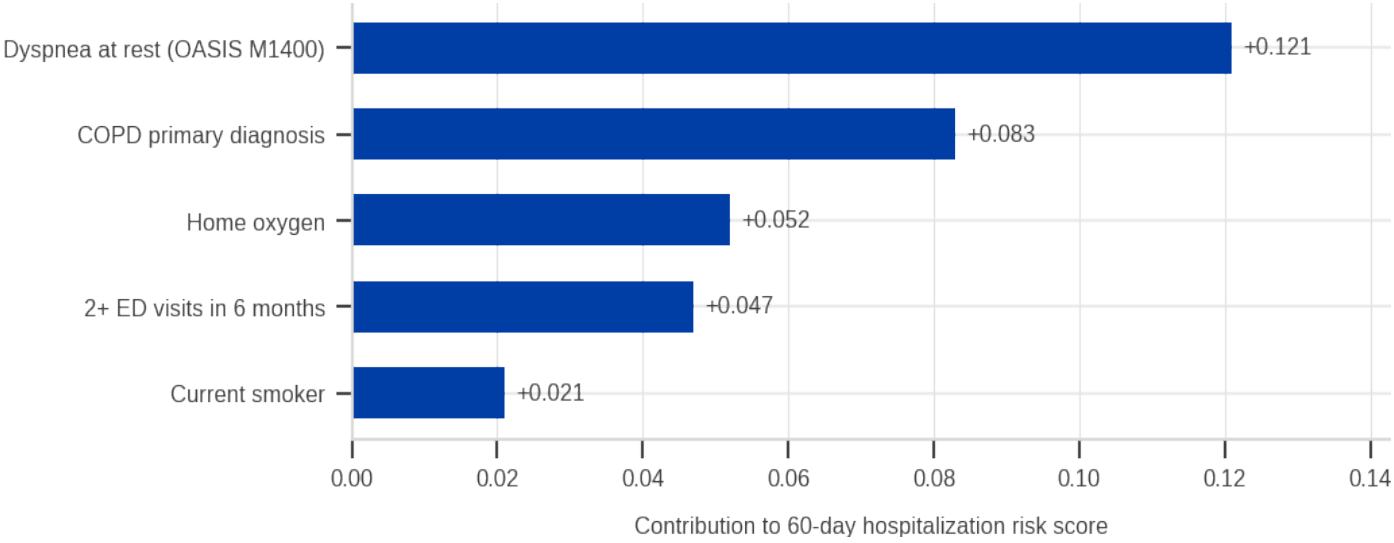
Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

Evidence for the CQI Patient Profile & Risk Summary and risk tile on page 7.

Episode timeline



Top drivers of 60-day hospitalization risk



FEEDBACK TIP Before rating **Risk tier and drivers** on **page 24** (fillable), check: the top drivers are clinically plausible for this patient; the prior inpatient stay and OASIS items match the chart.

CQI ANALYSIS

Vitals trajectory: action window

Weekly readings turned into a trajectory, an urgency and an action window

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

Evidence for the CQI Vitals Trajectory Summary on page 7.

WEEKLY VITALS
0 vitals days

TRAJECTORY
Insufficient data

URGENCY
No vitals

ACTION WINDOW
Obtain vitals at the next visit

What CQI saw

No vitals recorded for this episode.

What CQI flags

Monitoring gap: confirm the vitals plan with the SN.

VITALS DAYS	DAYS SINCE LAST READING	NEEDING REVIEW	TRAJECTORY
0	-	0	Insufficient data

What CQI found, by measure

Measure	First	Latest	Change	CQI interpretation	Urgency
-				No vitals findings	

FEEDBACK TIP Before rating **Vitals + action window** on **page 24** (fillable), check: readings match the visit documentation (units, dates, values); the trajectory and action window are clinically appropriate; the trend charts on page 20 support what CQI saw.

CQI ANALYSIS

Patient goals

Goals met, goals not yet met, and therapy goals

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

PATIENT-STATED GOAL (care plan wording, unchanged)

"No patient-stated goal found in the care plan."

GOALS MET	NOT YET MET	DELAYED / MISSED	THERAPY GOALS (PT/OT/ST)
0	0	0	0

Goals not yet met

Goal	Disc.	CQI status	Evidence found
None			

Goals met

Goal	Disc.	CQI status	Evidence found
None			

Therapy goals (PT / OT / ST)

Goal	Disc.	CQI status	Evidence found
None			

FEEDBACK TIP Before rating **Goals met / not met** on **page 24** (fillable), check: goal wording and status match the care plan; 'met' goals have documented evidence; therapy goals match PT / OT / ST plans.

CQI ANALYSIS

Care plan gaps: what to add

Interventions missing per goal, assessments or vitals to add, and visit goals to add

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

Interventions missing per goal

Care-plan goal	Missing intervention	Disc.
None identified		

Assessments or vitals to add

Assessment / vital	Type	Why it is needed
None identified		

Visit goals to add to meet patient goals

Disc.	Visit goal	Supports patient goal
None identified		

Medications, supplies and equipment linked to goals

Item	Type	Supports goal	CQI alignment check
Not yet compared			

FEEDBACK TIP Before rating **Care plan gaps to add**, **Medications and supplies** on **page 24** (fillable), check: each missing item is truly missing (not just undocumented); suggested visit goals are measurable and fit the patient's goal; medications and supplies match the current med profile.

Care network and discharge referrals

Primary care, CenterWell Primary Care (CW PCO) and CenterWell Pharmacy (CW Rx)

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

Refer to CenterWell Primary Care (CW PCO)?

Consider

Nearest CW PCO clinic 14.8 mi away (within 20 mi); confirm travel / transport.

Refer to CenterWell Pharmacy (CW Rx)?

Already enrolled

Patient uses CW Rx; confirm discharge medication list is shared.

Care network

Item	Status	Detail
Primary care provider (PCP)	No PCP on record	
Last PCP visit	Unknown	
Enrolled in CW PCO	No	
Enrolled in CW Rx	Yes	
Plan eligible (MA)	Yes	
Nearest CW PCO clinic	14.8 mi	CenterWell Senior Primary Care - Lenoir
CW PCO clinic within 10 miles	No	0 clinic(s)
CW PCO clinic within 20 miles	Yes	1 clinic(s)
Active medications	8	

How the referral indicator is set

- **CW PCO:** Refer if not enrolled, plan-eligible and a clinic is within 10 miles (stronger when there is no PCP). Consider if the nearest clinic is 10-20 miles away. Not indicated if none within 20 miles or not eligible.
- **CW Rx:** Refer if not enrolled, plan-eligible and taking 5 or more active medications. Already enrolled: share the discharge medication list.
- **Check eligibility** appears when plan eligibility is unknown. Indicators are prompts for the discharge conversation, not orders.

FEEDBACK TIP Before rating **Care network + referrals** on **page 24** (fillable), check: PCP and enrollment match the member record; the referral indicator makes sense for this patient; the patient or caregiver agrees before any referral.

SECTION 2

Evidence

The records behind the findings, and how CQI used them

Pages 14-22

These pages show the **operational records** CQI used, organized for this patient: the same data you would find in the EMR and standard reports. Use them to **confirm or challenge** the findings in CQI analysis. The last page explains **how CQI produced** those findings: methods, data sources and known limits.

Each page ends with a feedback tip that links back to the finding it supports.

In this section

Patient context and 485 profile — Demographics, living situation, profile — 15

Diagnoses and procedures — Codes, onset and exacerbation — 16

OASIS — Start of care vs. latest — 17

Visits: ordered, delivered, scheduled — Weekly grid by discipline — 18

Missed care and events — Missed visits, alerts, logs, inpatient — 19

Vitals trends — Charts against patient limits — 20

Assessments, interventions, adverse events — What was done at visits — 21

How CQI produced this — Methods, data sources, limits — 22

EVIDENCE

Patient context

Who the patient is: demographics, diagnoses, living situation and 485 / OASIS profile

Evidence for the CQI Patient Profile & Risk Summary on page 7.

Patient context

Age band / sex	75-79 / M	**Clinical group**	MMTA_RESPIRATORY
Primary diagnosis	COPD with acute exacerbation (J44.1)	**Comorbidities**	Not found
Living situation	Not found	**Admission type**	New Admission
Acuity level / first visit	Level 1; first visit DAY+1		

Clinical profile (485 / OASIS)

Comorbidity adjustment	Low	**Functional impairment**	High
Prognosis	Guarded	**Rehab potential**	Fair
Functional limitations	Endurance, dyspnea with minimal exertion	**Activities permitted**	Up as tolerated, +1 adl assist
Mental status	Oriented	**Allergies**	NKA
Nutritional requirements	Regular diet		

FEEDBACK TIP Before rating **Profile & Risk Summary** on **page 24** (fillable), check: diagnoses, comorbidities and living situation match the chart; allergies, mental status and activities permitted are current on the 485.

EVIDENCE

Diagnoses and procedures

Primary and secondary diagnoses, onset or exacerbation, and procedures this episode

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

CQI summary

No summary was generated for this run.

DIAGNOSES	PROCEDURES	ONSET / EXACERBATION	BODY SYSTEMS
2	0	1	2

Diagnoses

Code	Description	Body system	Clinical group	Onset / exacerbation
J44.1	**Primary:** COPD with acute exacerbation	Respiratory System	MMTA_RESPIRATORY	Exacerbation (DAY-2)
Z99.81	Dependence on supplemental oxygen	Factors Influencing Health		

Procedures

Code	Description	Clinical group	Date
No procedures recorded this episode			

FEEDBACK TIP Before rating **Diagnoses, wounds, OASIS** on **page 24** (fillable), check: diagnosis codes and the primary diagnosis match the 485 / plan of care; onset vs. exacerbation is coded correctly; the procedures listed belong to this episode.

EVIDENCE

OASIS

Key OASIS items at start of care and the latest assessment, with changes highlighted

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

CQI summary

No summary was generated for this run.

ITEMS CAPTURED	KEY RISK ITEMS	CHANGED SINCE SOC	ASSESSMENTS
0	0	0	0

OASIS items (key risk items first)

Item	Section	Description	Answer
No OASIS found for this episode			

FEEDBACK TIP Before rating **Diagnoses, wounds, OASIS** on **page 24** (fillable), check: answers match the OASIS in the EMR; changed items reflect a real change in the patient; M1033 risk factors are addressed in the care plan.

EVIDENCE

Care delivery

Ordered vs. delivered vs. scheduled visits by discipline and week

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

Evidence for the CQI Care Delivery Summary and Delay Impact on page 8.

Weekly visit frequency

Discipline	Wk 1	Wk 2	Wk 3	Wk 4	Wk 5	Wk 6	Wk 7	Wk 8	Wk 9	Total	Left / sched.
SN	3/3	3/3	1 of 2	1 of 2	1 of 2	0 of 1	0 of 1	0 of 1	0 of 1	6/6	10 / 3
PT	1/2	2/2	0 of 2	2 of 2	-	-	-	-	-	3/4	4 / 2

Weeks 1-2: delivered/ordered (green = met, plum = short). Weeks 3-9: scheduled of ordered.
Light blue rule = CQI run date. Left / sched. = ordered visits remaining vs. visits on the calendar.

FREQUENCY STATUS	SCHEDULE STATUS	ACUITY WINDOW MISSED	MISSED-CARE RECORDS
Unknown	**Unknown**	**No**	**1**

FEEDBACK TIP Before rating **Care Delivery Summary**, **Delay Impact Explanation** on **page 24** (fillable), check: ordered frequencies match the 485 / plan of care; delivered counts match the visit record; the future schedule is confirmed in the scheduling system today.

EVIDENCE

Missed care and events

Missed or outstanding visits, inpatient events, vitals alerts and client log notes

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

Evidence for the CQI Missed Care Summary on page 8.

Missed and outstanding care

When	Discipline	Visit	Reason / evidence	Status
DAY+14	**PT**	PT subsequent visit	Patient hospitalized	**Missed**

Inpatient events, vitals alerts and client log notes

When	Type	Detail	Source / follow-up
DAY+12	**Vitals alert**	SpO2 87% on 2L below parameter (88%)	**No physician notification found**
DAY+13	**Inpatient event**	ED visit, COPD exacerbation; treat and release	**Source: inpatient events**
DAY+14	**Client log note**	Daughter called: patient in ED overnight, discharged home	Source: client logs

FEEDBACK TIP Before rating **Missed Care Summary** on **page 24** (fillable), check: missed visits and reasons match the visit record and client logs; alerts flagged in plum had no documented physician notification.

EVIDENCE

Vitals trends

Each measure against the patient's limits, by episode day

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

Evidence for the CQI vitals action window on page 10.

No vitals found for this episode

This may be a monitoring or documentation gap. Confirm with the SN before conference.

FEEDBACK TIP Before rating **Vitals + action window** on **page 24** (fillable), check: readings match the visit documentation (units, dates, values); limits match the patient's ordered call parameters.

EVIDENCE

Care delivered

Assessments and interventions performed, adverse events and visit narrative themes

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

Assessments performed

No summary was generated for this run.

Assessment	Disc.	Count	Last day
None recorded			

Interventions performed

No summary was generated for this run.

Intervention	Disc.	Linked care-plan goal	Count	Last day
None recorded				

Adverse event summary

DAY+13 ED visit (Treat and release): COPD exacerbation

Visit narrative themes

No visit narrative themes were generated for this run.

FEEDBACK TIP Before rating **Assessments + interventions**, **Adverse events + themes** on **page 24** (fillable), check: counts match the visit documentation; the summaries describe what was actually done, not what was ordered; every adverse event is listed and correctly classified.

EVIDENCE

How CQI produced this summary

Methods, data sources and definitions: what to trust and what to check

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

From data to findings

1 Predict

Production 60-day hospitalization risk model scores high and rising-risk patients each week and explains top drivers.

2 Compare

Rules compare ordered, scheduled and delivered visits by discipline and week, timing windows and vitals limits.

3 Read

Language models read goals, care plans, notes and logs to draft the summaries on pages 7-8.

4 Check

AI drafts are checked by a panel of LLM judges and adjudicated. Clinician feedback from this pilot will guide improvements.

Data sources

Home Health operational data on the CW Lighthouse AI environment: admissions and episodes, visits, 485 plan of care and orders, vitals, assessments, interventions, OASIS, inpatient events, events, client logs, goals, notes, diagnoses and procedures, referrals, PDGM periods, supplies, medications, care-plan tasks, wounds.

Model outputs: CWHH 60-day transfer-to-inpatient risk scores and risk drivers.

Definitions

High / rising risk	High: top 25% of weekly risk scores. Rising: weekly score increasing versus prior runs.
Frequency status	on_track, minor_shortfall, behind or significantly_behind versus ordered visits to date.
Schedule status	Whether remaining ordered visits are on the calendar (e.g. no_future_schedule, needs_change).
Risk raised by care gaps	CQI judgment that missed or delayed care materially increased risk.
Delay clinical impact	Estimated clinical impact of timing delays: low, moderate or high.
Nearing discharge decision	Active episode within 14 days of the episode end date.
Urgency	Within 24 hrs, this week, or next visit - a recommendation for the team, not an order.

Known limits

- Findings reflect data available at the run date; documentation lag can look like a care gap.
- Missing goals or vitals may be a documentation gap rather than a care gap - check the chart.
- Recommendations are drafts for clinical review.

Questions or corrections

CW Data Science & Applied AI - HH Team | For more information visit go/lighthouse

SECTION 3

Your input

Fillable on screen: feedback, decision, notes

Pages 23-25

Your review makes CQI better. Rate each finding, record the conference decision and add notes **on screen**, then save and return the PDF. Answers are collected automatically.

Open in Adobe Acrobat Reader, Microsoft Edge or Chrome to fill in.

In this section

Clinician feedback 24
Rate each finding

Conference decision and notes 25
Decision, owner, follow-ups

YOUR INPUT

Clinician feedback (fillable)

Complete on screen, save and return. One form per patient; answers are collected automatically.

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

How to complete: open this PDF in Adobe Acrobat Reader, Microsoft Edge or Chrome, click the circles and boxes, type comments, then save (Ctrl+S) and return it to the CW Data Science & Applied AI - HH Team. **Tip:** rate at least the findings you discussed. For *Partly* or *Disagree*, tick an error type and add one line saying what was wrong - that is what improves CQI.

CQI finding	Pages	Agree	Partly	Disagree	Error type D I C N	Comment (what was wrong or missing)
At a glance + CQI flags	4	☐	☐	☐	☐ ☐ ☐ ☐	
Summary page + actions	5	☐	☐	☐	☐ ☐ ☐ ☐	
Profile & Risk Summary	7, 15	☐	☐	☐	☐ ☐ ☐ ☐	
Risk tier and drivers	9	☐	☐	☐	☐ ☐ ☐ ☐	
Vitals + action window	7, 10	☐	☐	☐	☐ ☐ ☐ ☐	
Care Delivery Summary	8, 18	☐	☐	☐	☐ ☐ ☐ ☐	
Missed Care Summary	8, 19	☐	☐	☐	☐ ☐ ☐ ☐	
Delay Impact Explanation	8, 18	☐	☐	☐	☐ ☐ ☐ ☐	
Data Gaps Found	8	☐	☐	☐	☐ ☐ ☐ ☐	
Goals met / not met	11	☐	☐	☐	☐ ☐ ☐ ☐	
Medications and supplies	12	☐	☐	☐	☐ ☐ ☐ ☐	
Assessments + interventions	21	☐	☐	☐	☐ ☐ ☐ ☐	
Adverse events + themes	21	☐	☐	☐	☐ ☐ ☐ ☐	
Care plan gaps to add	12	☐	☐	☐	☐ ☐ ☐ ☐	
Diagnoses, wounds, OASIS	16, 17	☐	☐	☐	☐ ☐ ☐ ☐	
Care network + referrals	13	☐	☐	☐	☐ ☐ ☐ ☐	

Error type: D = data missing I = data incorrect C = clinical interpretation N = not relevant for this patient

Conference outcome (tick all that apply)

☐ Plan updated ☐ Visits rescheduled ☐ MD notified ☐ No change needed ☐ Refer to Data Science

How useful was this booklet for conference?

☐ 1 Not useful ☐ 2 ☐ 3 ☐ 4 ☐ 5 Very useful

What should CQI add, remove or change?

Reviewer name

Role / discipline

Date

YOUR INPUT

Conference decision and notes (fillable)

Record the team's decision here. Saved with the feedback form.

Patient 9000231 | Episode 7000512 | Branch 0412 | Day 19 of 60

Conference decision

☐ Agree with CQI ☐ Agree with changes ☐ Disagree ☐ Escalate to MD

Action owner	Due date	Reviewed by

Discussion notes

Follow-ups and owners

Anything else for the CQI team